Gmail Pınar Özdemir Yaşar <pozdemiryasar@gmail.com>

Fwd: TJAR-2026-305 - (2853) Revise (major revisions)

1 mesaj

Nergis ÖZTÜRK <dr.nergisozturk@gmail.com> 14 Ekim 2026 18:01
Alıcı: Uzm Dr Pınar Özdemir Yaşar HÜTF Anestezi <pozdemiryasar@gmail.com>

Abla TJAR dan revizyon maili gelmiş

İleti başlangıcı:

Kimden: Turkish Journal of Anaesthesiology and Reanimation <tjar@manuscriptmanager.net>
Konu: TJAR-2026-305 - (2853) Revise (major revisions)
Tarih: 14 Ekim 2026 09:43:51 GMT+3
Kime: dr.nergisozturk@gmail.com
Yanıt Adresi: Zekeriyya Alanoglu <zekeriyya_69@yahoo.com>

Manuscript: TJAR-2026-305 - (2853) - Airway Management Challenges in Cervical Diffuse Idiopathic Skeletal Hyperostosis: Insights from Two Distinct Anatomical Presentations
Date submitted: 2026-09-25

Dear Dr. Öztürk

Thank you very much for submitting the above manuscript to Turkish Journal of Anaesthesiology and Reanimation.

Your manuscript has now been evaluated by external reviewers and members of the editorial board. While we certainly find merit in your manuscript, a number of points of criticism were raised and we therefore regret to inform you that your manuscript is not acceptable for publication in its present form. If you feel that you can adequately address the criticisms outlined in the reviewer reports below, we would be willing to reconsider a thoroughly revised version of your manuscript.

When you submit a revised version, please enclose:

1. a point-to-point reply to the reviewers' comments that addresses each comment of each reviewer. Please be as specific as possible in your response to the reviewer(s).
2. a revised version of the manuscript with changes made in bold, underlined or highlighted (adding page and line numbers)
3. a clean revised version (without line numbers).

We look forward to receiving the revised version within 3 months and thank you, again, for submitting your manuscript to Turkish Journal of Anaesthesiology and Reanimation.

Sincerely,

Dr. Zekeriyya Alanoglu, MD, DESA

zekeriyya_69@yahoo.com
Turkish Journal of Anaesthesiology and Reanimation

Reviewer 1 report:
Comments to authors
General comments
The authors report two patients with cervical diffuse idiopathic skeletal hyperostosis (DISH) who underwent anterior osteophytectomy, and they contrast airway compromise at the hypopharyngeal and cervicothoracic levels. The subject is of clear relevance to anaesthesiologists, and the comparison of two anatomical levels of involvement is a potentially instructive approach. In its present form, however, the case descriptions do not provide sufficient anaesthetic detail to substantiate the lessons drawn, and several statements in the Discussion and Conclusion extend beyond what can reasonably be inferred from two cases.
Title and Abstract
1. The expression “catastrophic management challenges” is not commensurate with the clinical course reported; a more measured formulation such as “significant airway management challenges” is suggested.
2. The description of preoperative computed tomography as “essential” should be moderated. A formulation such as “Review of preoperative imaging, particularly computed tomography, may assist in identifying sites of airway encroachment that are not apparent on bedside examination and in tailoring the anaesthetic strategy accordingly” would be preferable.
Case Presentation: Case 1
1. The induction agents, the neuromuscular blocking agent and the timing of its administration relative to confirmation of mask ventilation, the preoxygenation technique, and the lowest recorded oxygen saturation should be reported.
2. For each intubation attempt, the device, blade type, laryngeal view obtained and adjuncts employed (stylet, bougie) should be specified.
3. The authors should indicate which tracheal tube sizes could not be passed and the anatomical level at which resistance was encountered.
4. The type of laryngeal mask airway should be specified, together with the rationale for selecting a size 3 device in a patient weighing 73 kg.
5. The measures taken to optimise mask ventilation (oropharyngeal airway, two-handed technique) before it was judged unsuccessful should be described.
7. The diagnostic basis for DISH should be stated (for example, the Resnick criteria and the presence of thoracic involvement). As the patient is a 47-year-old woman with a 10-year history of generalised pain who also required discectomy, the authors should indicate whether ankylosing spondylitis and other spondyloarthropathies were excluded.

Case Presentation: Case 2
1. The "severe tracheal narrowing" described at C7–T1 is not adequately documented. The authors are asked to provide an axial computed tomography image at the level of maximal narrowing, with the trachea and the compressing osteophyte marked. It would also be helpful to report the anteroposterior and transverse diameters of the tracheal lumen at this level (in mm), together with the corresponding measurements at an unaffected level, so that the degree of narrowing can be expressed as a percentage.
2. Mask ventilation was easy and a 7.5 mm tracheal tube was placed at the first attempt. This seems difficult to reconcile with severe tracheal narrowing. Could the authors please clarify whether any resistance was felt as the tube was advanced?
3. Osteophytes were resected from C3 to C6, whereas the tracheal compression was described at C7–T1. Could the authors please clarify how the compression at this lower level improved after surgery?
4. The authors should state whether the patient had dyspnoea, stridor or positional symptoms, and whether a flow–volume loop was obtained.
5. The anaesthetic details requested for Case 1 should likewise be provided for this patient, including the dates of previous anaesthetics.
Discussion
1. Avoidance of neck hyperextension and “specialized endoscopic guidance” are presented as key elements of management, but neither is mentioned in the case presentations. These measures should either be described there or omitted from the Discussion.
2. The proposal of a “radiological airway clearance protocol” for all patients with DISH may be difficult to support on the basis of two cases. The authors might consider a more modest recommendation, for example that anaesthesiologists review the available imaging themselves in symptomatic patients.

3. A paragraph addressing the limitations of the report should be added.
Figures
1. Arrows and vertebral level labels should be added to all figures.
Other
1. The keyword “Diffucult” on the title page is misspelled.
2. Either “Forestier disease” or “Forestier’s disease” should be used consistently throughout.
3. The abbreviation OPLL is defined more than once.
4. The authors are asked to confirm adherence to the CARE reporting guidelines.

Reviewer 2 report:
Comments to authors

Dear Authors,
Thank you for this valuable case report. I would like to offer several suggestions that may help to further strengthen the manuscript:
Case 1 represents an anticipated difficult airway. The patient presented with a Mallampati class III airway, a mouth opening of two finger breadths, restricted cervical mobility, hypopharyngeal compression on computed tomography, ossification of the posterior longitudinal ligament at the C2–C3 level, and dysphagia that had acutely worsened over the preceding 10 days. Could the authors clarify why awake tracheal intubation was not chosen in this patient? Was the patient's panic disorder a contributing factor in this decision? Including this point in the Discussion would considerably enhance the educational value of the manuscript. Furthermore, was a pre-formulated written airway strategy (Plan A–D) in place, and were front-of-neck access preparation and otorhinolaryngology support available? Providing further details regarding the preoxygenation technique would also be helpful.
In Case 2, the postoperative computed tomography is reported to show resection of the osteophytes between C3 and C6, whereas the tracheal compression at the C7–T1 level is described as having "regressed." This anatomical discrepancy should be explained or corrected.
The phrase "careful patient positioning" in the Abstract is not reflected in the case descriptions. We recommend that this aspect be addressed in the Case Presentation section.
Finally, given that the conclusions are based on only two cases, we suggest that the proposal of a standardized "radiological airway clearance" protocol for all patients with Forestier's disease be moderated to a recommendation that such an approach "should be considered."
Yours sincerely,

ONE TIME QUICK LINK SIGN IN TO THE JOURNAL: https://www.manuscriptmanager.net/sLib/v4/quicksign.php?paramScreen=U15E1IoU+p0jVHVsj/KMCDQq9QlKN4bGAJfJlW65Uk5K553+M/FCw8ZZ5GJ1/Mx3